

Socket reconstruction is a surgical procedure used to rebuild and repair an anophthalmic socket. We use orbital implants, mucous membrane grafts and dermis fat grafts to restore the lost volume and to reconstruct the fornix so that a customized prosthetic eye can be placed.

Surgeries that are commonly done include:

- Eversionation with ball implant
- Enucleation with ball implant.
- Dermis fat graft with for fornix formation sutures.
- Mucous membrane grafts are used to deepen shallow fornix by replacing the lost conjunctiva.
- Post eversionation and enucleation, a customized prosthetic eye can be placed after approximately 6 weeks.
- Most dermis fat graft and mucous membrane grafts, a customized prosthetic eye can be placed after approximately 2 months.

- The dermis fat graft is harvested from the abdomen or buttocks.
- Mucous membrane graft is harvested from the buccal cavity or lips.
- PMMA ball implants are commercially available.
- Add this tag just before the mucous membrane grafts are used to deepen sentence.

Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.

- Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.
- Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.
- Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.

Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.

- Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.
- Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.
- Epiphora refers to an excessive tearing, usually of the external or weeping of the external eye, not the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye. It is not the weeping of the external eye, which the weeping itself is a symptom of the eye.

Epiphora is the medical terminology for excessive watering from the eyes.

Causes:

- Blockage in the tear drainage pathway;
- Narrow or a blocked punctum.
- Upper or lower Canalicular stenosis or blockage.
- Common canalicular blockage.
- Canaliculitis
- Nasolacrimal duct blockage.
- Eyelid abnormalities:
- Lid laxity
- Ectropion
- Entropion
- Epiblepharon
- Euryblepharon
- Trichiasis congenital or acquired.
- Ocular and Ocular surface abnormalities:
- Allergy
- Infection
- Inflammation
- Corneal or ocular surface foreign body
- Pterygium
- Conjunctivochalasis, that is loose conjunctiva.
- Dry eye: dryness due to environmental issues or secondary to Sjogrens syndrome, post LASIK etc can cause reflex watering.
- Meibomian gland dysfunction can also cause mild to moderate watering.
- Excessive production of tears can cause watering.

Treatment:

- Tear drainage pathway blockage:
- Mainly surgical.
- Nasolacrimal duct blockage: external DCR or Endoscopic DCR with stenting.
- Canalicular blockage:
- Trephination with stenting
- Trephination plus DCR with stenting.
- Punctal stenosis:
- 3 snip procedure with or without stenting
- Canaliculitis: Medical treatment or Canaliculotomy with curettage.
- Lid abnormalities: surgical.
- Conjunctivochalasis: surgical, tightening of the loose conjunctiva.
- MGD: Medical management, Hot fomentation, IPL therapy.
- Dry eye: Tear substitutes, and treatment of the underlying condition.
- Ocular infection and allergy: medical management.

Oculofacial trauma

Oculofacial trauma involves physical injury to the eyelid and adnexal structures, eye, face, bony orbit surrounding the eye. Most commonly caused by assault, road traffic accident, self fall and trauma with a blunt or sharp object.

Signs and symptoms:

- Redness of the eye, subconjunctival haemorrhage.
- Swelling around the eye.
- Extraocular movement restriction.
- Sometimes diminution or loss of vision.
- Double vision
- Lid and oculofacial lacerations with or without skin loss.
- Restriction of jaw movements.
- Compartment syndrome, that is tightness around the eye or frozen globe.

Diagnosis and management.

- A thorough ocular examination to rule out any intraocular injury or optic nerve injury.
- CT scan of the face and head to look for facial fractures, muscle entrapment and to rule out any head injury.
- Stabilize the patient.
- Paediatric orbital fractures are also called white eyes fractures and require immediate surgery to release the entrapped muscle and to restore normal eye movement.
- Adult orbital floor fracture causing double vision, muscle entrapment and severe exophthalmos require surgical intervention with an implant. Best results are obtained if the surgery is done within the first 2 weeks.
- ZMC fractures and severely comminuted orbitofacial fractures require open reduction and internal fixation with screws, plates and implants. A multidisciplinary approach involving an Oculoplasty surgeon and a maxillofacial surgeon is often required.
- Lid, canalicular and oculofacial lacerations require reconstructive surgery.
- Traumatic optic neuropathy require immediate intervention with high dose intravenous steroids.

Blepharoplasty:

Blepharoplasty is a surgical procedure which improves the appearance of the eyelids. Blepharoplasty can be combined with other surgical procedures like brow lift, internal blephary and face lift.

- In upper lid blepharoplasty, we usually excise a thin strip of skin and the pretarsal fat through a lid crease incision.
- In lower lid blepharoplasty, we reposition the pretarsal fat, transconjunctally followed by excision of a thin strip of skin.
- It can be performed on both the upper and lower lid.
- Blepharoplasty is usually performed under local anaesthesia.

Preoperative preparation:

- Baseline blood investigations.
- Stop taking aspirin, blood thinners, multivitamins, herbal medications as these can increase the risk of bleeding.
- Quit smoking.

Who can undergo blepharoplasty?

- Patients having loose, sagging upper lid skin with lateral hooding blocking vision.
- Upper lid puffiness due to fat prolapse.
- Lower lid bags.
- Excess lower lid skin.
- Patients with realistic expectations are good candidates for blepharoplasty surgery.

Post operative recovery:

- The swelling and the ecchymosis gradually resolves over a period of 6 weeks to 2 months.
- Suture removal is done after 10 days.

Conclusion:

- Blepharoplasty is predominantly a cosmetic procedure, which restores confidence and in certain cases improve the field of vision.

Lid tumors:

- Lid tumors are abnormal growths on the upper and lower lid.
- Lid tumors can be benign or malignant.
- Most lid tumors benign or malignant, need surgery.
- For malignant lid tumors a wide excision with 4 mm clear margins is required followed by lid reconstruction surgery.
- The lid surgery to be done depends on the size of the defect post excision.